

VISUAL HOSPITAL-FAMILY HANDOUT

The Hospital Discharge & Medicare Quick Guide

A plain-English starting point for families before discharge, rehab, home support, long-term care, or a confusing bill.

Written from a healthcare-worker perspective by Andrew Ciccarelli, RN, BSN. Educational only. Verify with official sources, the plan, facility, billing office, SHIP, or a qualified professional.

1

Status

2

Payer

3

Approval

4

Documents

5

Next call

1

STATUS

What is the official status?

2

PAYER

Which coverage path applies?

3

APPROVAL

Covered, pending, denied, or ending?

4

DOCUMENTS

What notice, EOB, or MSN proves it?

5

NEXT CALL

Who can verify in writing?

REMEMBER

The right first question prevents the wrong next step.

1

CORE IDEA

Use this guide to ask the first right questions before discharge, rehab, home support, long-term care, or paying a confusing bill.

OK Pick the problem

STAT

HOSPITAL STAY

What is the patient's official status?

REH

REHAB NEXT

What skilled need is documented?

HOME

HOME SUPPORT

What service or equipment is ordered?

LTC

LONG-TERM HELP

Is this skilled care or daily living help?

BILL

CONFUSING BILL

Does it match the MSN, EOB, or plan paperwork?

OK First move

1

Identify the payer.

2

Ask for the rule or document.

3

Get the answer in writing.

4

Verify before signing or paying.

SOURCE NOTE

Orientation page. Use Medicare.gov, Medicaid.gov, CMS.gov, official state Medicaid agencies, SHIP, plan documents, and facility or billing-office documents for situation-specific verification.

AVOID COSTLY ASSUMPTIONS



The 5 Misunderstandings

Five traps that make families overpay, panic, or miss appeal windows.

VERIFY

Whether coverage is approved, limited, pending, or denied

RISK IF MISSED

Assumptions can hide patient responsibility and appeal deadlines.

1 Do not assume

2 Ask for proof

3 Confirm possible cost

REMEMBER

Need, coverage, approval, and payment are four different things.



CORE IDEA

Most confusion comes from five expensive assumptions. Slow down when any of these show up.

RISK Watch out

OK

NEED CARE

Need does not equal payment.

OK

COVERED

Covered does not always mean free.

REH

REHAB

Short-term skilled care is not long-term custodial care.

STAT

HOSPITAL BED

Overnight does not always mean inpatient.

OK

DOCTOR ORDER

A recommendation is not the same as payer approval.

ASK Better question

OK

What rule applies?

DOC

What document proves it?

OK

Who can verify it?

BILL

What could the patient owe?

SOURCE NOTE

Supported by Medicare.gov coverage, cost, skilled nursing facility, long-term care, and Medicare Advantage comparison guidance.

PROGRAM MAP



Medicare vs Medicaid

Medicare and Medicaid are not interchangeable.

VERIFY

Medicare, Medicaid, both, or neither

RISK IF MISSED

The wrong program assumption can create the wrong coverage expectation.

1 Identify coverage

2 Check state rules

3 Verify provider fit

REMEMBER

First identify the payer, then ask which rule applies.



CORE IDEA

Medicare and Medicaid answer different questions. Some people have both, but the exact category still matters.

OK Compare

MED MEDICARE

Federal health insurance tied to age, disability, or certain conditions.

MCD MEDICAID

State-run assistance under federal rules for eligible people.

OK DUAL ELIGIBILITY

The person has Medicare and some form of Medicaid help.

RISK KEY WARNING

Having both does not mean every provider, service, facility, or bill is covered.

ASK Ask

MCD Does the person have Medicare, Medicaid, both, or neither?

REH Is this medical care, skilled care, custodial care, or long-term support?

MCD Which state Medicaid agency applies?

BILL Is the help full Medicaid or limited Medicare-cost assistance?

SOURCE NOTE

Supported by Medicare.gov, Medicaid.gov eligibility policy, Medicaid.gov long-term services and supports, CMS Medicare-Medicaid coordination resources, and Medicaid.gov Medicare-Medicaid enrollee resources.

COVERAGE PATH



Original Medicare vs Medicare Advantage

Same Medicare umbrella, very different process.

VERIFY

Original Medicare, Medicare Advantage, and any extra coverage

RISK IF MISSED

Network, authorization, and cost rules may change the next step.

1 Name the plan type

2 Check network/approval

3 Ask what is owed

REMEMBER

Original Medicare and Medicare Advantage can feel very different at the point of care.



CORE IDEA

Original Medicare and Medicare Advantage are two ways to receive Medicare. The process can feel very different.

OK

Compare

MED

ORIGINAL MEDICARE

Usually broader provider access if the provider takes Medicare.

AUTH

MEDICARE ADVANTAGE

Private Medicare-approved plans may use networks, service areas, referrals, and prior authorization.

OK

EXTRA COVERAGE

Part D, Medigap, Medicaid, retiree coverage, or other help may change costs.

RISK

KEY WARNING

Neither path means every cost disappears.

ASK

Ask before services

AUTH

Is this Original Medicare or Medicare Advantage?

HOME

Is the provider, facility, pharmacy, or supplier approved?

AUTH

Is prior authorization required?

OK

What is the covered-service out-of-pocket limit?

BILL

What could the patient owe?

SOURCE NOTE

Supported by Medicare.gov How Medicare Works, Medicare.gov Compare Original Medicare and Medicare Advantage, Medicare.gov Costs, and CMS Medicare managed care appeals guidance.

STATUS CHECK



Inpatient vs Observation

Where the patient sleeps is not always the billing status.

VERIFY

Official hospital status and when inpatient admission began

RISK IF MISSED

Observation time can affect downstream Original Medicare SNF rules.

1 Ask current status

2 Confirm admission time

3 Get notices

REMEMBER

Overnight in the hospital does not automatically mean inpatient.



CORE IDEA

A patient can sleep in the hospital and still not be formally admitted as inpatient.

OK Why it matters

STAT

INPATIENT

May affect downstream Original Medicare skilled nursing facility rules.

OK

OUTPATIENT

Hospital care without formal inpatient admission.

OK

OBSERVATION

Outpatient hospital status used while the hospital evaluates care needs.

REH

SNF RISK

Observation or emergency room time before inpatient admission does not count toward the Original Medicare qualifying inpatient stay.

ASK Ask now

STAT

What is the official hospital status right now?

STAT

When did inpatient admission begin, if it began?

REH

Does this affect rehab or skilled nursing facility coverage?

DOC

Has any required observation notice been provided?

AUTH

How does the Medicare Advantage plan handle this?

SOURCE NOTE

Supported by Medicare.gov Skilled Nursing Facility Care and CMS Medicare Outpatient Observation Notice / Beneficiary Notices Initiative. Exact CMS MOON wording and timing should be rechecked before final public release.

DISCHARGE CHECKPOINT



Before Discharge

Separate the care plan from the payment plan before leaving.

VERIFY

Destination, payer, approval status, and backup plan

RISK IF MISSED

Leaving without the payment path can create avoidable confusion later.

1 Where next?

2 Who pays?

3 What is the backup?

REMEMBER

A safe discharge plan still needs a verified payment plan.



CORE IDEA

Before discharge, separate the care plan from the payment plan.

OK Discharge path

1

STATUS

Inpatient, outpatient, or observation?

2

DESTINATION

Home, home health, rehab, SNF, long-term care, or other?

3

PAYER

Original Medicare, Medicare Advantage, Medicaid, or another plan?

4

APPROVAL

Covered, denied, pending, or waiting on authorization?

5

BACKUP

What happens if coverage ends or home is unsafe?

ASK

Ask before agreeing

REH

What skilled need is documented?

HOME

What services, equipment, medications, and follow-up are ordered?

BILL

Which payer is expected to pay?

BILL

What could the patient owe?

DOC

What written notice or appeal instructions should we review?

SOURCE NOTE

Supported by Medicare.gov skilled nursing facility, home health, durable medical equipment, costs, appeals, Medicare Advantage comparison, and CMS observation-notice resources.

REHAB REVIEW

R

Rehab and Skilled Nursing Facility Care

Short-term skilled care has rules, reviews, and deadlines.

VERIFY

Skilled need, facility fit, prior authorization, and appeal window

RISK IF MISSED

Coverage can end or be denied faster than families expect.

1 Document skill

2 Confirm facility

3 Track appeal deadline

REMEMBER

Rehab coverage depends on skilled need, rules, facility fit, and timing.

R

CORE IDEA

Medicare may cover short-term skilled rehab or skilled nursing facility care when rules are met. It is not automatic.

OK Timeline

1

HOSPITAL

Confirm status and skilled need.

2

TRANSFER

Confirm facility and payer rules.

3

REVIEW

Ask whether coverage is approved, denied, pending, or ending.

4

AFTER SKILLED CARE

Plan for home support, outpatient therapy, private pay, Medicaid/LTSS, or other help.

ASK Ask

REH

What skilled service is being ordered?

STAT

Is this SNF, inpatient rehab, home health, or outpatient therapy?

MED

Is the facility Medicare-certified or in-network?

AUTH

Is prior authorization required and approved?

RISK

What is the appeal deadline if denied or ending?

SOURCE NOTE

Supported by Medicare.gov Skilled Nursing Facility Care, Medicare.gov Costs, Medicare.gov Compare Original Medicare and Medicare Advantage, and Medicare.gov appeals guidance. Current-year dollar amounts are intentionally not listed in this quick guide; verify them before release and use the full guide or site calculator for updated figures.

CARE SETTING SORT



Home Health, Equipment, and Long-Term Care

Home health, equipment, and custodial care are different questions.

VERIFY

Whether the need is skilled care, equipment, custodial help, or long-term support

RISK IF MISSED

Daily living help is often the coverage gap families discover late.

1 Sort the need

2 Check agency/supplier

3 Plan long-term help

REMEMBER

Skilled help, equipment, and daily living help follow different coverage paths.

CORE IDEA



Home health, equipment, and long-term daily help are different coverage questions.

OK Sort the need



HOME HEALTH

Usually part-time or intermittent skilled services when rules are met.



EQUIPMENT

The supplier and item must meet Medicare or plan rules.



CUSTODIAL CARE

Help with bathing, dressing, toileting, meals, supervision, or daily living.



LONG-TERM SUPPORT

Medicaid, state programs, community resources, or private pay may become the bigger question.

ASK Ask



Is this skilled care, equipment, custodial care, or long-term support?



Is the agency Medicare-certified or plan-approved?



Is the supplier enrolled in Medicare or approved by the plan?



Does the supplier accept assignment?



Should Medicaid/LTSS or professional guidance be explored?

SOURCE NOTE

Supported by Medicare.gov Home Health Services, Medicare.gov Durable Medical Equipment Coverage, Medicare.gov Long-Term Care, Medicaid.gov LTSS, and Medicaid.gov eligibility policy.

BILL CHECK



Bills, EOBs, and Medicare Summary Notices

Match the bill against the processed claim before paying.

VERIFY

Bill, date of service, MSN/EOB, and patient responsibility

RISK IF MISSED

A bill can be premature, miscoded, duplicated, or misunderstood.

1 Match documents

2 Ask why owed

3 Correct or appeal

REMEMBER

Do not pay a confusing bill until it matches the processed claim story.

CORE IDEA



Before paying a confusing bill, match the bill to the Medicare Summary Notice, EOB, or plan paperwork.

DOC

Compare the documents

BILL

PROVIDER BILL

What someone is asking the patient to pay.

DOC

MSN OR EOB

What Medicare or the plan processed.

OK

PATIENT RESPONSIBILITY

The amount listed as owed after processing.

ASK

NEXT CALL

Billing office, plan, Medicare, SHIP, or financial assistance office.

BILL

Ask before paying

OK

What date of service is this?

BILL

Which provider or facility is billing me?

AUTH

Has Medicare or the plan processed the claim?

OK

Why is this patient responsibility?

BILL

Should this be appealed, corrected, rebilled, or screened for financial assistance?

SOURCE NOTE

Supported by Medicare.gov Medicare Summary Notice, Medicare.gov appeals, Medicare.gov Costs, and HealthCare.gov glossary sources for deductible, copayment, coinsurance, allowed amount, and out-of-pocket maximum.

CALL SCRIPTS



Scripts and Next Steps

Slow the conversation down and get the answer in writing.

VERIFY

Who answered, what they said, what document supports it, and the deadline

RISK IF MISSED

Unwritten answers are hard to use when coverage or billing changes.

1 Call the right place

2 Use the script

3 Write down the answer

REMEMBER

The best question is the one that gets a clear, documented answer.



CORE IDEA

Use the scripts to slow the conversation down and get the answer in writing.

OK Phone scripts

STAT

HOSPITAL

What is the patient's status, discharge destination, skilled need, and payer rule?

OK

FACILITY

Are you Medicare-certified, in-network, or plan-approved?

AUTH

PLAN

Is authorization required, approved, denied, pending, or partial?

BILL

BILLING

Can you compare this bill against the MSN, EOB, or plan paperwork?

OK Next step

1

Use the full guide for deeper chapters and worksheets.

2

Use the Medicare care cost tool for updated cost estimates.

3

Use the EOB-to-bill checker before paying confusing medical bills.

4

Contact SHIP, Medicare, Medicaid, the plan, the facility, the billing office, or a qualified professional for person-specific decisions.

SOURCE NOTE

This page summarizes the verification workflow from the full guide. Final QR codes and public download links should only be added after destination testing and public-release review.

OPTIONAL TRUST LAYER



Endnotes and Source Map

Official sources used to keep the short guide grounded without turning it into a textbook. **This page is optional during urgent decision-making.**

- 1 Medicare.gov — How Medicare Works:
<https://www.medicare.gov/basics/get-started-with-medicare/medicare-basics/how-does-medicare-work>
- 2 Medicare.gov — Compare Original Medicare and Medicare Advantage: <https://www.medicare.gov/basics/get-started-with-medicare/get-more-coverage/your-coverage-options/compare-original-medicare-medicare-advantage>
- 3 Medicare.gov — Costs: <https://www.medicare.gov/basics/costs/medicare-costs>
- 4 Medicare.gov — Long-Term Care: <https://www.medicare.gov/coverage/long-term-care>
- 5 Medicare.gov — Skilled Nursing Facility Care: <https://www.medicare.gov/coverage/skilled-nursing-facility-care>
- 6 Medicare.gov — Home Health Services: <https://www.medicare.gov/coverage/home-health-services>
- 7 Medicaid.gov — Long-Term Services and Supports: <https://www.medicare.gov/coverage/long-term-services-supports/index.html>
- 8 Medicaid.gov — Eligibility Policy: <https://www.medicare.gov/coverage/skilled-nursing-facility-care>
- 9 Medicare.gov — Medicare Summary Notice: <https://www.medicare.gov/basics/forms-publications-mailings/mailings/claims-and-appeals/medicare-summary-notice>
- 10 Medicare.gov — Appeals: <https://www.medicare.gov/providers-services/claims-appeals-complaints/appeals>
- 11 CMS.gov — Medicare Managed Care Appeals and Grievances: <https://www.cms.gov/medicare/appeals-grievances/managed-care>
- 12 CMS.gov — Medicare Outpatient Observation Notice / Beneficiary Notices Initiative: <https://www.cms.gov/medicare/forms-notices/beneficiary-notices-initiative/moon>
- 13 Medicare.gov — Durable Medical Equipment Coverage: <https://www.medicare.gov/coverage/durable-medical-equipment-dme-coverage>
- 14 HealthCare.gov glossary sources: deductible, copayment, coinsurance, out-of-pocket maximum, and allowed amount.
- 15 Medicaid.gov — Seniors & Medicare and Medicaid Enrollees: <https://www.medicare.gov/coverage/durable-medical-equipment-dme-coverage>